

Disability and Shame Special Issue Forum: Research Article

Dealing with Shame: Saudi Mothers' Responses to Their Children's Diagnosis of Autism Spectrum Disorder

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Abstract: Autism is a disorder affecting individuals' academic, communication, and social development. The findings of this qualitative study report 15 Saudi mothers' responses to their children's diagnosis of autism spectrum disorder and provide insights into mothers' experiences and responses to stigma in the context of the Kingdom of Saudi Arabia.

Keywords: Autism Spectrum Disorder; Stigma; Mothers' Perceptions

Introduction

Individuals diagnosed with autism spectrum disorder (ASD) have difficulties when communicating and often show repetitive interests and behaviors. Therefore, mothers of children with this condition have encountered many problems when raising them. How ASD manifests depends on the chronological age and developmental level of the children. Symptoms of ASD include delayed toilet training, temper tantrums, self-injurious behavior, aggression, impulsivity, and hyperactivity (Duarte, Bordin, Yazigi, & Mooney, 2005). On the other hand, mothers of children diagnosed with ASD are more likely to experience depression, higher pessimism about their children's future, and reduced quality of life (Cappe, Wolff, Bobet, & Adrien, 2011; Johnson, Frenn, Feetham, & Simpson, 2011). According to previous research, it is evident that parents of infants diagnosed with ASD experience more parenting stress relative to other individuals (Hayes & Watson, 2013), indicating that having children with ASD can strongly impact the health and well-being of parents. Consequently, the degree of parental stress is proportional to the severity of children's behaviors and symptoms of autism (Davis & Carter, 2008; Ingersoll & Hambrick, 2011; Lyons, Leon, Phelps, & Dunleavy, 2010). When children manifest poor behavior and severe symptoms, their parents subsequently experience higher parental stress.

Furthermore, parents of children diagnosed with ASD often feel shame. For example, they may experience disapproval and criticism from neighbors as they are perceived as being unable to control their children (Gray, 1993; Mak & Kwok, 2010). Due to this stigma, children diagnosed with ASD are perceived as behaving inappropriately of their own volition. As a result, many individuals think that parents are to be blamed for using ineffective parenting styles when raising their children (Fernandez & Arcia, 2004). Consequently, mothers might feel they are not utilizing appropriate interventions or treatments, specifically when their friends advise them about various other treatments or interventions they should use (Miller, Schreck, Mulick, & Butter, 2012). Additionally, mothers may blame themselves for not being able to help their children develop properly (Kuhn & Carter, 2006). They might

even blame themselves for being the reason for their children's diagnosis. Therefore, these experiences of self-blame, shame, and guilt are the major causes of anxiety, depression, and parenting stress (Cappe et al., 2011; Mak & Kwok, 2010).

The shame, blame, and stigma experienced by individuals with disabilities and their families are reflected in the moral and medical models of disability where fault is attributed to the individual or his/her family and not to society (Olkin, 2002). Olkin (2002) proposes in the moral model of disability, that disability "is a defect caused by moral lapse of sins, failure of faith, evil, test of faith...brings shame to the person with the disability and his or her family" (p. 133). The moral model results in individuals "being ostracized from family and community, having profound shame, having to hide disability symptoms or the person with a disability" (Olkin, 2007, p. 6).

Review of the Literature

This study examined mothers' responses to the cultural stigma associated with their children's diagnosis of autism and the role institutions play. Individual interviews with mothers allowed for an examination of these women's experiences navigating issues of stigma related to autism in the Kingdom of Saudi Arabia (KSA). Goffman's (1963) theory of stigma as a theoretical framework underpinning this research review and stigma is examined in relation to culture, paying particular attention to the role that the Saudi culture plays in the lives of mothers whose children have been diagnosed with autism.

Culture

Culture plays a role in shaping the way individuals see, think, and understand the world. It comprises "a dynamic yet stable set of goals, beliefs, and attitudes shared by a group of people" (Gurung, 2006, p. 172) and even though some beliefs are fluid, "the general level of culture stays stable because individuals change together" (Gurung, 2006, p. 4).

As members of a community, the lives of individuals with ASD are generally determined by the culture in which they are immersed. Families and communities do the best they can to help individuals with ASD function as productive members of society and live as normally as possible, guided by their cultural beliefs and level of understanding. The beliefs held by members of a community regarding the potential and treatment of individuals with autism differ from one culture to another and can change over time (Ravindran & Myers, 2012). Culture influences key aspects of the diagnosis and treatment process, and affects the likelihood that individuals seek help, what treatments to use, available resources, and the relationships between families and professionals. Daley (2002) suggests treatment approaches that work and are culturally accepted in the West may not be the same approaches accepted and used to treat individuals with ASD in other parts of the world. For instance, in the West, known causes of autism are increasingly attributed to science and various treatments that are available which include behavioral, cognitive, pharmaceutical, sensory, relational, vitamins, and diet therapy. However, in other cultures, the causes of autism might be regarded as "'Karma,' 'Allah's will,' or 'the will of God'" and may be treated with non-scientific

remedies such as acupuncture and/or herbal medicine (Ravindran & Myers, 2012, p. 311).

ASD and Stigma

Within the scientific community, autism is viewed as a disorder marked by abnormal social skills stemming from an incurable medical condition (Martin, 2012). Over the years, the negative stigmatization and social perceptions associated with ASD have grown due to the individualized nature of the syndrome and a lack of understanding of the symptoms (Martin, 2012). Historically, one of the most common social challenges faced by individuals with autism is a feeling of isolation as they grow older, due to increased segregation and rejection from peers, combined with an increase in self-awareness (Martin, 2012). An individual's level of isolation is dependent on key factors such as environment, personality, and intensity of impairment, all being impacted by the level of stigmatization received from others in the surrounding environment. Isolation and rejection are further exacerbated by the fact that the social behaviors of individuals with autism are perceived as different from the norm, or deviant. As such, since the behaviors of those with autism do not meet the expectations of what is appropriate, they are often ignored and avoided by the normative groups.

Goffman's Theory of Stigma

Goffman's (1963) framework is useful for examining Saudi mother's perceptions of their experiences with stigma associated with their children's autism. Particularly when discussing the societal stigma associated with having certain characteristics, Goffman sheds light on how communities stigmatize individuals that display certain traits that are deemed undesirable. This is true for individuals with autism in Saudi Arabia, where the culture still stigmatizes such individuals.

In *Stigma: Notes on the Management of Spoiled Identity*, Goffman (1963) describes the different types of stigma and how they impact the identity of those who are labeled undesirable by society. Goffman refers to those who are not forced to live under a perceived stigma, or "do not depart negatively from the particular expressions at issue" as "*normals*" (p. 5). This perspective is in line with how society views individuals who have been stigmatized, as well as, behaving the opposite of how normal society should act, despite how damaging these thought processes are to the relationships between the stigmatized and the general population.

Goffman (1963) identifies three separate forms of social stigma that are essential to conceptualizing how stigma impacts members of a society, whether individuals are, or may be perceived through this ever-present lens. The first form of social stigma involves physical abnormalities that are referred to with harsh, unsympathetic words. Goffman writes these "abominations of the body -- the various physical deformities" are enveloped with harsh judgment and are prevalent despite the physical differences occurring without the person's will or desire (p. 4).

The second form of social stigma is associated with characteristics or behavioral traits that bring negative connotations despite often being no direct connection between the trait and